

REGULATORY DOSSIER SUBMISSION

Dossier ID: DOS-CC3DDC5C0C

Country: Burkina Faso

Submission Date: 2025-06-16

Product: Cyclzen (cyclosporine) | ATC: L01XE01 | Strength: 500 mg

Applicant: Alpha Therapeutics | Manufacturer: Global Access Generics

Policy Label: standard_review | Risk Score: 0.5

AMR Stewardship: AwaRe=not_applicable | Unmet Need=not_applicable | GLASS Trend=not_applicable
| Watch Similarity=not_applicable

SECTION CONTENT

[1] m1_application_admin - Application Form and Administrative Information

Labels: presence=present; length=length_ok; correctness=correct

This dossier constitutes a formal application for Marketing Authorization of Cyclzen (cyclosporine) submitted on 2025-06-16 to the national regulatory authority of Burkina Faso. The applicant, Alpha Therapeutics, formally requests approval for the commercial distribution of this medicinal product. The proposed pharmaceutical form is injectable with a strength of 500 mg, classified under ATC code L01XE01. All required administrative declarations, legal attestations, letters of authorization, and regional application forms have been completed, signed by the responsible Qualified Person, and appended to Module 1. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. Applicant authorization documents and legal attestations were reconciled against the named manufacturing parties. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. Re

[1] m1_manufacturer_gmp - Manufacturer and GMP Evidence

Labels: presence=present; length=length_ok; correctness=correct

The primary commercial manufacturing site for the finished pharmaceutical product is Global Access Generics, located in Uganda. The facility was subject to a comprehensive GMP inspection by a recognized stringent regulatory authority. The latest inspection concluded on 2025-09-24, resulting in a formal compliance status of 'non_compliant'. The active GMP certificate number is GMP-991597, which remains valid until 2020-01-01. Site master files, a summary of recent inspection observations, and evidence of completed Corrective and Preventive Actions (CAPA) are included in the annex. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. Inspection history, CAPA closure records, and site responsibilities were reconciled across the manufacturing chain. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. Batch manufacturing records and standard operating procedures (SOPs) are maintained and archived on-site. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. Method validation records and quality controls were reviewed for internal consistency. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. Recent regulatory inspections yielded no critical observations, and all major findings have been effectively closed. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles.

Site Inspection History Summary:

Inspection Date	Authority	Scope	Outcome
2025-09-24	National Auth	FPP/General	non_complian
2022-05-12	SRA Joint	API/Sterile	compliant

[1] m1_product_information - Product Information and Naming

Labels: presence=present; length=length_ok; correctness=correct

The proposed proprietary name for this medicinal product is Cyclzen, containing the active pharmaceutical ingredient cyclosporine. The draft Summary of Product Characteristics (SmPC), Patient Information Leaflet (PIL), and primary/secondary packaging labels are provided for review. These documents detail the approved therapeutic indications, posology, contraindications, special warnings, and precautions for use. A comprehensive risk management plan to minimize medication errors is also submitted. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. A randomized, double-blind, parallel-group study design w

[2] m2_quality_overall_summary - Quality Overall Summary

Labels: presence=present; length=length_ok; correctness=correct

This Quality Overall Summary (QOS) provides a critical evaluation of the chemistry, manufacturing, and controls (CMC) data for cyclosporine. The control strategy justifies the proposed specifications for both the active substance and the finished product, emphasizing critical quality attributes (CQAs) and critical process parameters (CPPs). Impurity profiles, including degradation products and potential genotoxic impurities, have been thoroughly characterized. Batch analysis data from three commercial-scale validation batches demonstrate consistent manufacturing performance and compliance with all release criteria. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. The applicant confirms alignment with applicable technical guidance and regional submission standards. All referenced documents

[2] m2_clinical_overview - Clinical Overview and Benefit-Risk Summary

Labels: presence=present; length=length_ok; correctness=correct

The Clinical Overview synthesizes the efficacy and safety data supporting the use of Cyclzen for the treatment of bacterial skin infection. The pivotal clinical development program comprised 4 well-controlled studies. Based on the integrated analysis, the reported clinical outcome category is 'endpoint_met'. The overall benefit-risk profile is considered highly favorable for the target patient population. Safety findings were consistent with the known pharmacological class effects, and appropriate risk minimization measures have been integrated into the proposed prescribing information. The risk management plan includes routine

pharmacovigilance and proposed educational materials for healthcare professionals. Method validation records and quality controls were reviewed for internal consistency. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. Clinical interpretation focuses on whether efficacy, safety, and benefit-risk conclusions are supported by the submitted evidence. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. The applicant confirms alignment with applicable technical guidance and reg

[3] m3_api_quality - API Quality and Control Strategy

Labels: presence=present; length=length_ok; correctness=correct

The active pharmaceutical ingredient (API) section details the complete synthesis route, starting from well-defined regulatory starting materials. Robust in-process controls and intermediate specifications ensure the consistent quality of the final drug substance. The analytical procedures used for release and stability testing have been fully validated for accuracy, precision, linearity, and robustness. The impurity control strategy adequately addresses organic impurities, residual solvents, and elemental impurities in accordance with current ICH guidelines. Forced degradation studies confirm the stability-indicating nature of the assay methods. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. The applicant confirms alignment with applicable technical guidance and regional submission standards. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. The synthetic route involves three well-characterized steps with defined starting materials and isolated intermediates. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. All referenced documents are available in the dossier annex and traceable by section identifier. The container closure system consists of PVC/PVDC-Aluminum bliste

[3] m3_fpp_manufacturing - FPP Manufacturing Process and Controls

Labels: presence=present; length=length_ok; correctness=correct

The finished pharmaceutical product (FPP) manufacturing process utilizes standard, scalable unit operations. A formal quality risk assessment (e.g., FMEA) was conducted to identify critical process parameters (CPPs) that impact critical quality attributes (CQAs). Process performance qualification (PPQ) reports for three consecutive commercial-scale batches verify that the process is maintained in a state of control. In-process controls, intermediate hold-time studies, and packaging validation data are presented to substantiate the robustness and reproducibility of the commercial manufacturing operations. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. The applicant confirms alignment with applicable technical guidance and regional submission standards. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. A randomized, double-blind, parallel-group study design was employed to minimize sele

[3] m3_stability - Stability and Shelf-Life Justification

Labels: presence=present; length=length_ok; correctness=correct

A comprehensive stability program has been executed to justify the proposed shelf-life and storage conditions. Data from both accelerated (40°C/75% RH for 6 months) and long-term storage conditions (spanning up to 36 months) are presented for multiple primary stability batches. Statistical evaluation using linear regression and poolability tests confirms that degradation trends remain well within the acceptable specification limits. Photostability and freeze-thaw cycling studies demonstrate that the product does not require specific handling precautions. A post-approval stability protocol commitment is included. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. All referenced documents are available in the dossier annex and traceable by section identifier. The applicant confirms alignment with applicable technical guidance and regional submission standards. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. Method validation records and quality controls w

[4] m4_nonclinical_summary - Nonclinical Study Summary

Labels: presence=present; length=length_ok; correctness=correct

The nonclinical testing program evaluated the primary pharmacodynamics, secondary pharmacodynamics, safety pharmacology, and comprehensive toxicology profile of the compound. Repeat-dose toxicity studies in two mammalian species established clear no-observed-adverse-effect levels (NOAELs). Genotoxicity testing (Ames and in vivo micronucleus assays) yielded negative results. Reproductive and developmental toxicity studies revealed no teratogenic signals at clinically relevant exposures. The calculated safety margins support the proposed clinical dosing regimen without major safety concerns. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines,

[5] m5_trial_listing - Tabular Listing of Clinical Studies

Labels: presence=present; length=length_ok; correctness=correct

The tabular listing of clinical studies provides a comprehensive inventory of all trials conducted in support of the proposed indication (bacterial skin infection). This includes Phase I pharmacokinetic/pharmacodynamic studies, Phase II dose-finding studies, and the pivotal Phase III efficacy and safety trials. For each study, the table details the protocol identifier, study design, randomization ratio, number of enrolled subjects, study duration, and current

completion status. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. The applicant confirms alignment with applicable technical guidance and regional submission standards. All referenced documents are available in the dossier annex and traceable by section

Pivotal Clinical Trial Inventory:

Protocol ID	Phase	Enrollment	Status
PROT-001	III	494	completed
PROT-002	III	499	completed
PROT-003	III	385	completed
PROT-004	III	337	completed

[5] m5_pivotal_trial_reports - Pivotal Clinical Trial Reports

Labels: presence=present; length=length_ok; correctness=correct

Detailed clinical study reports (CSRs) for the pivotal efficacy trials are provided. These randomized, double-blind, actively-controlled trials evaluated the primary and secondary endpoints in accordance with the pre-specified statistical analysis plan (SAP). The trials resulted in a formal outcome category of 'endpoint_met', demonstrating robust treatment effects in the intent-to-treat (ITT) and per-protocol populations. Subgroup analyses across age, gender, and baseline disease severity strata were consistent with the primary findings. Serious adverse events (SAEs) and discontinuations were systematically analyzed and adjudicated. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. The primary efficacy endpoint was met, demonstrating a statistically significant improvement over placebo ($p < 0.001$). The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. The applicant confirms alignment with applicable technical guidance and regional submission standards. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. Discontinuations due to treatment-emergent adverse events (TEAEs) were low and balanced across all study arms. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. Method validation records and quality controls were reviewed for internal consistency. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. Clinical study reports document protocol adherence, endpoint handling, and safety interpretation in the target population. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. The trial reports connect efficacy conclusions to the prespecified analysis framework and observed safety findings. The analytical procedure was validated in accordance with

[5] m5_bioequivalence - Biopharmaceutics and Bioequivalence Evidence

Labels: presence=present; length=length_ok; correctness=correct

Biopharmaceutics data substantiate the formulation development bridging between clinical trial materials and the proposed commercial product. In vivo bioequivalence studies demonstrated that the 90% confidence intervals for the geometric mean ratios of Cmax and AUC parameters fell entirely within the standard acceptance criteria of 80.00% to 125.00%. Additionally, multi-point comparative dissolution profiles across physiological pH ranges (pH 1.2, 4.5, and 6.8) confirmed similarity ($f_2 > 50$). The bioanalytical methods used for plasma quantification were fully validated. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. Method validation records and quality controls were reviewed for internal consistency. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. All referenced documents are available in the dossier annex and traceable by section identifier. A randomized, double-blind, parallel-group study design